



GUIDELINE

Support Strategies for CAMHS Staff

Scope (Staff):	All staff
Scope (Area):	CAMHS-wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Aim

This document outlines the support strategies available for all CAMHS staff to address the psychosocial hazards in the workplace resulting from critical incident/s as well as the regular and prolonged exposure to the traumatic experiences of others.

Risk

The psychological and physical wellbeing of CAMHS staff may be adversely affected by exposure to psychosocial hazards resulting in:

- increased illness and absenteeism;
- burnout;
- compassion fatigue;
- reduced motivation, productivity and commitment;
- decreased team cohesion and communication;
- high turnover; and/or
- decrease in staff morale and engagement.

All of the above can impact on the safety and quality of care provided to children and families.

Background

As a trauma informed service, CAMHS is committed to promoting the diverse range of support strategies available to staff to address the psychosocial hazards inherent in the workplace, specifically following a critical incident/s or the regular and prolonged exposure to the traumatic experiences of others. CAMHS recognises the importance

of providing this support for staff experiencing adverse effects deemed to be beyond a normal stress response, and in the prevention of burnout, compassion fatigue and/or vicarious trauma.

For information on the identification, reporting, review, management and evaluation of clinical incidents, including sentinel events, refer to the [CAHS Clinical Incident Management Policy](#). In addition, the [CAMHS Sentinel Event Management Toolkit](#) was developed to assist CAMHS Service Managers and Heads of Service in accessing the appropriate sources of information to refer to following a sentinel event.

Definitions

Burnout: An emotional and behavioural response to prolonged exposure to occupational stressors.¹ Symptoms and/or outcomes of burnout include;

- cynicism;
- depersonalisation;
- emotional exhaustion; and/or
- inefficacy.

Compassion fatigue: An array of psychological, behavioural, cognitive and physical responses resulting from prolonged engagement with traumatised and suffering individuals. It occurs when emotional boundaries become blurred and the caregiver (staff member) unconsciously absorbs the distress, anxiety, fears and trauma of the patient.¹ Symptoms and/or outcomes of compassion fatigue include;

- avoidance;
- depression;
- exhaustion;
- intrusion;
- frustration; and/or
- hypervigilance.

Critical Incident: is any event or series of events which causes a person to experience strong emotional reactions which have the potential to interfere with their ability to function either during the event or later. Critical incidents may be a single event or be a series of events over an extended period. Critical incidents that can trigger the need for support include, but are not limited to;

- being involved in a clinical or non-clinical incident, near miss, complaint or claim, especially one involving serious harm to a patient or patient suicide;

- being involved with a major incident involving a large number of casualties or deaths, especially incidents involving children;
- providing witness statements or information for CAHS investigations into an incident, near miss, complaint or claim;
- being interviewed or appearing as a witness in an external investigation such as a coronial inquiry, police investigation, or registering body hearing;
- an unexpected death or a patient/consumer, visitor or colleague;
- a suicide attempt / medical emergency;
- a serious injury to a colleague;
- a violent incident involving significant injury;
- a major fire;
- a hostage or terrorist situation;
- a bomb scare; and
- chronic stress situations (the accumulation of a series of incidents over a period of time).

Debriefing: is a structured session for the group of people involved in a critical incident, with the aim of reducing impact and providing information on the availability of appropriate professional support.

Defusing: an intervention that is a shorter, less formal version of a debriefing to provide immediate support to staff affected by a critical incident.

Vicarious trauma: the psychological consequences of working with people who have experienced trauma. It is a state of physical, emotional and mental exhaustion caused by long term involvement in emotionally demanding situations.² Symptoms of vicarious trauma can be different for each person but potential impacts on staff can be;

- intrusive imagery including repeated and persistent flashbacks or nightmares, images and recollections of traumatic events;
- negative changes to the way in which a staff member views the world;
- disrupted personal and professional relationships resulting from feeling a loss of safety, trust, control, intimacy and esteem.
- problems with managing feelings;
- difficulties with decision making; and
- physical symptoms including fatigue, insomnia and nausea.

Key points

- Psychosocial hazards in the workplace are aspects of work and work situations which can lead to psychological harm.
- Staff need to be supported during and after exposure to critical incidents and traumatic experiences of others in the workplace, to minimise the impact on their psychological safety.
- Some roles within CAMHS have inherent psychosocial hazards and risks which need to be managed in order to prevent adverse effects such as burnout, compassion fatigue and vicarious trauma.
- Support strategies are not mandatory for staff but must be offered to those involved in critical incidents.

Types of Support Strategies

CAHS-wide

- Defusing, for staff affected by a critical incident, takes place as soon as practical after a critical incident (usually within one to four hours). For more information see the [CAHS Critical Incident Impact Management \(Debrief\) Policy](#).
- Debriefing, for the group of people involved in a critical event, must be arranged as soon as practicable, but preferably within 72 hours of the incident. For more information see the [CAHS Critical Incident Impact Management \(Debrief\) Policy](#).
- The [CAHS Employee Assistance Program \(EAP\)](#) can be accessed for formal debriefing or staff can self-refer and use the program for ongoing support as required.
- The [Aboriginal Employee Assistance Program](#) provided by Converge International is delivered by indigenous consultants who specialise in indigenous issues with an in-depth understanding of cultural sensitivities and required approaches.
- Professional development and training opportunities focussing on enhancing coping skills and resiliency are available through external training providers. For more information see the CAHS [Supporting Employee Performance and Development page](#) and [CAHS Learning and Development page](#) on HealthPoint.
- The [Pastoral Care Service](#) is a no-cost, confidential, emotional, spiritual or religious support service. For more information see [HealthPoint](#).
- CAHS offers a range of flexible work arrangements under the [CAHS Flexible Work Policy](#) which supports staff wellbeing.
- Leave entitlements, including personal leave can be accessed as discussed with the line manager and [CAHS HR](#) and in accordance with the relevant [industrial instrument](#).

- The [Wellbeing at CAHS page](#) on HealthPoint has resources and information on how to maintain a healthy lifestyle and self-care strategies.
- The [CAHS Work Health Safety and Wellbeing \(WHSW\)](#) team have additional information and resources on [Psychological Trauma on HealthPoint](#) and can also assist in accessing support services and other practical support as needed.

CAMHS

- Clinical Supervision (including cultural supervision for Aboriginal Mental Health Workers) is the process of two or more professionals (supervisor and supervisee/s) formally meeting to reflect and review clinical situations with the aim of supporting the employee in their clinical environment including in response to a challenging clinical encounter. For more information refer to the [CAMHS Clinical Supervision Policy](#).
- Multidisciplinary Team (MDT) meetings are an opportunity for the Multidisciplinary Team (MDT) to reflect on clinical practice, therapeutic relationships and any clinical incidents, including those involving restraint and seclusion (see [CAMHS Physical Restraint Policy](#) and [CAMHS Seclusion Policy](#)).
- CAMHS SSAMHS huddle is an opportunity for CAMHS Aboriginal Mental Health Workers to get together, share information and support each other in their roles.
- CAMHS directorate Business and Governance meetings provide an opportunity to discuss clinical issues and concerns.
- Line management meetings between staff and their line manager can be used to discuss and give consideration to current workload and the allocation of new clients (where applicable).
- Teams can include a standing agenda item on psychological safety into daily huddles, stand up meetings or handover meetings as a way of “checking in” and to reflect on any challenges in the workplace and for managers to identify whether further support may be required for staff.
- The [CAMHS Policies page on HealthPoint](#) has links to additional tools and resources including a Professional Quality of Life Self-Assessment tool (to assess compassion satisfaction and compassion fatigue) and a [Mental Health Self Care Wheel](#) developed by the ACT Government Community Services Directorate.

Roles and Responsibilities

All CAMHS staff

- Take reasonable care for their own psychological safety and health in the workplace in accordance with the [Work Health and Safety Act 2020 \(WHS Act\)](#) and the [WorkSafe Code of Practice: Psychosocial Hazards](#).

- Be aware of what constitutes a normal stress response to a critical incident/s and if they are experiencing prolonged, adverse effects consistent with burnout, compassion fatigue and/or vicarious trauma, to actively seek the most appropriate support available as outlined in this guideline.
- Recognise the value of, and participate in, internal and external reviews of critical incidents as required.

Managers

- Responsible for leading and promoting safe working practices within their teams including ensuring psychological safety of staff.
- Monitor the workplace for signs that staff may be adversely affected by a critical incident/s and/or experiencing burnout, compassion fatigue and/or vicarious trauma and then initiate and facilitate access to appropriate supportive strategies in collaboration with staff.
- Ensure staff are aware of all the support strategies available to them as outlined in this guideline and facilitate the take up of staff support strategies as required.
- Escalate psychological safety concerns to the Head of Department and/or Director if required.
- Monitor and review the need and effectiveness of staff support strategies.

Head of Department and Director

- Provide a safe system of work in which staff are not exposed to psychosocial hazards in the workplace as far as practicable in accordance with [Work Health and Safety Act 2020 \(WHS Act\)](#) and the [WorkSafe Code of Practice: Psychosocial Hazards](#).
- Address psychological safety concerns if escalated by a Manager.

Monitoring and Reviewing

- Monitoring of the psychosocial wellbeing of staff should occur through discussions at planned meetings as well as analysing data from:
 - hazard, incident and investigation reports;
 - consultation with Work Health and Safety representatives and teams;
 - direct observations (i.e. staff displaying symptoms of vicarious trauma, burnout and/or compassion fatigue as listed in this guideline).
- A standing agenda or discussion item on psychosocial hazards at planned meetings and WHS committee meetings helps to ensure that regular monitoring occurs.
- Incident reports and findings should be reviewed to identify trends and to determine whether additional support measures such as training should be provided to staff.

Related CAHS internal policies, procedures and guidelines
CAHS Clinical Incident Management Policy
CAMHS Sentinel Event Management Toolkit
CAHS Critical Incident Impact Management (Debrief) Policy
CAHS Flexible Work Policy
CAMHS Clinical Supervision Policy
CAMHS Physical Restraint Policy
CAMHS Seclusion Policy
References and related external legislation, policies, and guidelines
¹ Bywood P, McMillan J. Cumulative exposure to trauma at work. Phase 2: Prevention and intervention strategies (2019). Institute for Safety, Compensation and Recovery Research (ISCRR). Available from: 226_REP_ER_Cumulative-trauma-Phase-2-FINAL-15.11.2019.pdf (iscrr.com.au)
² Conrad, D. Secondary trauma and caring professionals: Understanding tis impact and taking steps to protect yourself (2011). The Link, 20(2): 1-5. Available from: ispcan_link_20.2.pdf
WA Health Awards and Agreements Library
Work Health and Safety Act 2020 (WHS Act)
WorkSafe Code of Practice: Psychosocial Hazards
Useful resources (including related forms)
CAHS Employee Assistance Program (EAP)
Aboriginal Employee Assistance Program
Supporting Employee Performance and Development page
CAHS Learning and Development page
Pastoral Care Service
Wellbeing at CAHS page

[Psychological Trauma \(health.wa.gov.au\)](https://health.wa.gov.au)

[CAMHS Policies page on HealthPoint](#)

This document can be made available in alternative formats on request.

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*Note: CAMHS Leadership Committee approved a 15-month extension of the review date to 14/12/2026.